

APPELLANT'S SCHEDULE OF MEDICAL DOCUMENTS RELIED UPON

Served on the Respondent with the outlines of evidence under direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

No report has been prepared for the purposes of this proceeding. The Appellant relies on the treating records and reports below, each of which is already held by the Respondent at the item of its amended List of Documents dated 14 August 2026 stated, except item 6, a copy of which is served with this schedule. Against each document is stated what it is relied upon for and what it is not relied upon for, so that its use at the hearing is clear. The pages relied upon follow at Tabs 1 to 7; pages not relied upon are omitted, and each tab sheet identifies them.

No.	Document, and where the Respondent holds it	Relied upon for	Not relied upon for
1	General-practice records, Our Medical Ashmore, 1 January 2023 to 1 July 2024 (Respondent's item 11; obtained by the Respondent under the Form 29 signed 4 July 2025).	Prior health: the entry of 16 November 2023 (Dr Nanayakkara) recording poor sleep with shift work, that the Appellant could not do shifts without a good sleep, no psychological illness such as depression or psychosis, and mood good, with melatonin and temazepam prescribed; the 2022 history and the referral renewed 16 May 2024. First presentation after onset: 28 June 2024 (Dr Slawinski) recording "stress at work" and "upset by people not following rules", reason for visit anxiety; and 1 July 2024 (Dr Hawes) recording "work stress", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies", and "causing anxiety". These entries are relied upon as the contemporaneous record of what was reported before any claim decision, dismissal or proceeding.	Entries unrelated to the injury. Private medical entries unrelated to the injury are redacted on the extracted pages in the served copy and marked as such.
2	Work capacity certificates of Dr Peter Hawes dated 1 July 2024, 11 August 2024 and 8 September 2024 (Respondent's item 7).	The stated date of injury, 18 June 2024; that the Appellant was first seen for this injury on 1 July 2024; certification of no functional capacity from 1 July 2024; and the referral to a psychiatrist recorded on 8 September 2024.	The recorded mechanism as a finding of fact. The events are proved by the notice to admit facts served 28 August 2026, not by the certificate.
3	Email from the practice of Dr Ravikumar Bangalore Krishnaiah to the Appellant, 24 October 2024 at 11:45 am, "Medications" (Respondent's item 9; Appellant's List item 22).	That on the morning of 24 October 2024, at the first consultation, the treating psychiatrist told the Appellant, and confirmed in writing at 11:45 am that day, that he was "suffering from psychological injury of Major Depressive Disorder with anxiety state"; that fluoxetine was increased to two capsules "from today" and Seroquel 25 mg commenced at night, "to restore basic needs- sleep, eating and routine". Read with item 2, it records the progression of the injury certified as "anxiety, stress" on 1 July 2024 to a depressive disorder by 24 October 2024.	The reference to preoccupation with the claim as a cause of the injury. The injury relied upon is that of 18 June 2024; the email records its state four months later.
4	Report of Dr Krishnaiah, Mind and Memory Service, 13 February 2025, prepared for QSuper (Respondent's item 10).	Diagnosis: Major Depressive Disorder with anxious distress (DSM-5 296.23). Severity and functional effect, and treatment (fluoxetine increased to three capsules daily; quetiapine 25 mg at night). The treating clinician's account of origin: "workplace stress stemming from issues with management and rostering"; that the issues "began approximately one year ago when a new manager was appointed"; pay "withheld or delayed for up to five months at a time, leading to significant financial stress"; and that "premature exposure to the workplace is more likely result in significant deterioration".	Attribution among the individual events at paragraphs 2 to 7 of the Appellant's outline of evidence; any matter after 24 October 2024 as a cause of the injury. The report was prepared for QSuper and is tendered as a treating record, not as a report prepared for this proceeding.
5	Clinical records of Dr Krishnaiah from 24 October 2024 (offered by the practice on 5 September 2026 and requested; not yet received).	What was reported at the first consultation and when; diagnosis and prescribing over time. To be served on receipt. If any direction is required for reliance on them, the Appellant will apply for it.	Nothing until received.
6	Employee Capability Checklist completed by Dr Day Hong Ma, 3 July 2026 (copy served herewith).	Current capacity and restrictions, and the continuing effect of the injury, including "symptom exacerbation on exposure to the identified workplace stressors".	Causation. It post-dates 1 July 2024 and is relied upon for effect and capacity only.
7	Review Decision 69983, reasons dated 24 October 2024, pages 17 and 26 to 27 (Respondent's item 4; contents admitted 18 February 2026).	That the Respondent's own review recorded Dr Hawes's statement to WorkCover of 2 September 2024 that work events were the sole cause, found "a personal injury of a psychological nature", found employment "was a significant contributing factor" as to factors 2, 3 and 4, and found the March 2024 rostering to be unreasonable management action.	As a binding determination. The hearing is de novo; the finding is relied upon as an admitted document.

The Appellant reserves the position as to any further report. If one is to be relied upon, directions will be sought before it is served.

Dated: September 2026 Cory Lea Shepherd, Appellant, self-represented

TAB 1

General-practice records, Our Medical Ashmore, 1 January 2023 to 1 July 2024

Where the Respondent holds it

Respondent's amended List of Documents item 11. Obtained by the Respondent under the Form 29 signed 4 July 2025. The complete record is Exhibit A5, sealed by the Commission on 13 March 2026; these are extracts of it.

Pages included

Pages 5, 6, 7, 10, 11, 12 and 13 of the 14-page practice export: the consultation of 16 November 2023 (Dr Nanayakkara); the consultations of 16 May 2024 (Dr Zhao, referral renewed) and 28 June 2024 (Dr Slawinski); the consultation of 1 July 2024 (Dr Hawes); the referral letter to Dr Amini of 16 May 2024 with the past medical history; the medical certificate of 28 June 2024; and the work capacity certificate of 1 July 2024.

Pages omitted, and why

Pages 1 to 4 (patient details, medication and prescription lists, consultations of 2023 for unrelated conditions); page 8 (Gold Coast University Hospital discharge letter of 6 June 2024, dental); page 9 (medical certificate of 3 July 2023, unrelated); page 14 (blank). None is relied upon. REDACTIONS: on source pages 5, 6 and 10, entries concerning private medical matters unrelated to the injury have been blacked out by the Appellant and are marked as such on the page. Nothing relied upon has been redacted. The Respondent holds the unredacted record at item 11.

Schedule row 1 states what this document is, and is not, relied upon for.

Recorded by: Dr Wali Kamali Visit date: 12/09/2023

Recorded on: 12/09/2023

CO sorethroat for few days
fever on & off

Examination:

temp 37.5

Ear, nose & throat:

No nasal discharge. No nasal obstruction. Red throat. Both tonsils enlarged. No pus on tonsils.

Reason for visit:

Sore throat

Actions:

Prescription printed: Klacid 250mg Tablet 1 Twice daily

REDACTED by the Appellant: private medical matter unrelated to the injury

Surgery consultation

Recorded by: Dr Priyal De Silva Nanayakkara Visit date: 16/11/2023

Recorded on: 16/11/2023

h/o

Poor sleep
Shift work

Takes melatonin 2 mg to help to go to sleep, soem time 4 mg, but does not help much

Cannot work/ do shifts if he does not get a good sleep.

Does not take them every day

No psychological illness such as depression/ psychosis.

mood good.

I did a script for Melatonin- 6mg(get from Compounding chemist)
Temaze prn only

Review interval:

progress.

Actions:

Dose of changed from to Before bed 6mg / nocte /prn --- Compounded..

Prescription printed: Temaze 10mg Tablet 1 Before bed p.r.n.

Prescription printed: Melatonin 5mg Tablet, modified release Before bed Once / nocte / prn

REDACTED by the Appellant: private medical matter unrelated to the injury

Surgery consultation

Recorded by: Dr Wali Kamali Visit date: 24/11/2023

Recorded on: 24/11/2023

For medical form
applying for his super
having multiple dental procedures
ad should ask the sp tpo do the form
pt pt not understanding at all
kepp asking to fill the form
form done

REDACTED by the Appellant: private medical matter unrelated to the injury

Surgery consultation

Recorded by: Dr William Zhao Visit date: 16/05/2024

Recorded on: 16/05/2024

renew referral to psychiatrist

REDACTED by the Appellant: private medical matter unrelated to the injury

Actions:

Letter to Dr Arash Amini printed.

Letter written to Dr Arash Amini re. 1.Specialist Referral Letter Ashmore.

Prescription printed: Melatonin 5mg Tablet, modified release Before bed 6mg / nocte /prn --- Compounded.

REDACTED by the Appellant: private medical matter unrelated to the injury

Surgery consultation

Recorded by: Dr Bogdan Slawinski Visit date: 28/06/2024

Recorded on: 28/06/2024

wants melatonin
to help to sleep

stress at work
upset by people not following rules

Examination:
well, alert, t 36,6

Reason for visit:
Anxiety

Actions:
Prescription printed: Melatonin 5mg Tablet, modified release Before bed 6mg / nocte /prn --- Compounded.
Letter printed.
Letter written re. 1. Medical Certificate Ashmore.

Surgery consultation

Recorded by: Dr Peter Hawes Visit date: 01/07/2024
Recorded on: 01/07/2024

work problems- ethical complaint about manager and director, they don't follow the law
been there 5 years
feels he is undervalued
they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies, not
allowed to do this
they don't listen to his complaints
all this is stressing him out, causing anxiety,

Reason for visit:
work stress

Actions:
Letter printed.
Letter written re. QLD Work capacity certificate.

Investigations:



Our Medical Ashmore

566 Olsen Avenue, Ashmore QLD 4214

HealthLink ourmedic | **Phone** 07 5647 5000

Fax 07 5616 8014

Email reception@ourmedicalashmore.com.au

A Cornerstone Health Pty Ltd Medical centre

Operated by

Wycombe Services Pty Ltd

ABN 40 613103 450

www.ourmedicalhome.com.au

16/05/2024

Dr Arash Amini

Evandale Practice

Suite 2, 1st Floor Evandale Place, 142 Bundall Road

Bundall 4217

Phone: (07) 5510 3122

Fax: (07) 5510 3144

Re: Mr Cory Lea Shepherd

Add: U207 W\l\ Marina Drive

Benowa 4217

DOB: 11/01/1991

Medicare No: 4330920095 1

Ph: 0422438627

Dear Arash,

Thank you for seeing Mr Cory Lea Shepherd for ongoing care and management.

Current medications are:

Azithromycin 500mg Tablet

2 Tablets stat.

Melatonin 5mg Tablet, modified release

Before bed 6mg / nocte /prn --- Compounded.

REDACTED by the Appellant: private medical matter unrelated to the injury

Past Medical History:

26/10/2022 ADHD
26/10/2022 Anxiety
28/06/2023 Infection

Allergies:

Nil known.

Yours faithfully,

Dr William Zhao
BMED,AMC, FRACGP
Provider No: 264389EW



Our Medical Ashmore

566 Olsen Avenue, Ashmore QLD 4214

HealthLink ourmedic | **Phone** 07 5647 5000

Fax 07 5616 8014

Email reception@ourmedicalashmore.com.au

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Operated by

Wycombe Services Pty Ltd

ABN 40 613103 450

www.ourmedicalhome.com.au

MEDICAL CERTIFICATE

28/06/2024

THIS IS TO CERTIFY THAT :

Mr Cory Lea Shepherd has a medical condition and will be unfit for Work/ Study/ Usual activity
from 27/06/2024 to 11/07/2024 inclusive.

Dr Bogdan Slawinski
General Practitioner, MBBS, FACEM
208300QJ

Work capacity certificate – workers' compensation

Form 132M — Version 1

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity to work. Consider the health benefits of work when certifying the patient's capacity.

Part A - Patient details

Name Cory Shepherd Date of Birth 11/01/1991
Mobile number 0422438627 Claim number S23LW142013 ☒ New claim ☐ Claim is report only
Occupation (if known) Admin worker Patient's employer Qld Health

Part B - Injury details

Date of examination 01/07/2024 Patient's stated date of injury 18/06/2024 Patient was first seen at this practice/hospital for this injury/disease on 01/07/2024

The patient is/was suffering from (List all work-related diagnoses. *If symptoms only, tick "Provisional diagnosis"*) ☐ Provisional diagnosis anxiety, stress

Patient's stated mechanism of injury harrasment at work

Is this consistent with your clinical findings?

☒ Yes ☐ Unclear

Describe mechanism in detail ongoing breaking of workplace rules by bosses, victimizing him
Pre-existing factors or condition
aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from 01/07/2024 to 04/08/2024 to be reviewed again on 04/08/2024 ☐ No further review

Treatment needs time away from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Name/discipline Details (specify)
Specialist/GP

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F. ☐ If suitable duties available, can return to some form of work from ☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)
stressed and anxious,

Estimated time to return to some form of work duties unknown

Estimated time to return to full duties unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)	Is functional ability affected by injury/condition?	Note any restrictions (if relevant)	What patient can do (if "yes" is selected)
Lower limb	No		
Upper limb	No		
Hand function	No		
Spinal function	No		
Cognition/psychosocial functioning	No		
Driving a car	No		
Operating machinery/heavy vehicle	No		
Manual tasks	No		
Other	No		

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

☐ I require a suitable duties program to be provided to me for approval

I have discussed injury requirements and return to work with the patient and ☐ Employer ☐ Insurer ☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name Peter Hawes

Email admin@ourmedicalashmore.com.au

Practice/hospital Our Medical Ashmore

Phone 0756475000

Date 01/07/2024

Postal Address

Signature

566 Olsen Avenue, Ashmore

QLD 4214

Further information www.worksafe.qld.gov.au/medicalsupport

All enquiries (medical/dental/nurse practitioner, patient, employer) 1300 362 128

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation insurer is authorised to collect the information on this form to process the claimant's application for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers or any other workers' compensation authority in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any other medical, allied health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 31 May 2016, pursuant to section

588 of the Act. © State of Queensland (Queensland Treasury) 2016



TAB 2

Work capacity certificates of Dr Peter Hawes, 1 July 2024, 11 August 2024 and 8 September 2024

Where the Respondent holds it

Respondent's amended List of Documents item 7.

Pages included

The certificate of 8 September 2024, from the Appellant's photograph of the signed original (the only copy the Appellant holds; the Respondent holds the original at item 7). The certificate of 1 July 2024 is reproduced at Tab 1 (bundle page 9; source page 13 of the practice export).

Pages omitted, and why

The certificate of 11 August 2024 is not held by the Appellant and is not reproduced; the Respondent holds it at item 7.

Schedule row 2 states what this document is, and is not, relied upon for.

Work capacity certificate

Consider the signs, symptoms, and test results you have observed.

Part A - Patient details

NAME: JILL A. DODD
DOB: 01/01/1991

Date of Birth: 11/01/1991

☐ New claim
☐ Claim is report only

Name	Conf Singhania	Claim number	805594930	patient's employer	Old Heam
Mobile number	0422438527				
Occupation (if known)	Admin worker				
Additional details					
					01/07/2024

Part 8 - Injury details	Date of examination	date of injury	diagnoses	anxiety, stress
11/06/2024 <td>18/06/2024 <td></td> <td></td> <td></td> </td>	18/06/2024 <td></td> <td></td> <td></td>			

The patient is/was suffering from (List all work-related provisional diagnoses) ☐ Provisional diagnosis if hospital only. Tick "Provisional diagnosis" if hospitalized at work related mechanism of injury.

Is this consistent with your clinical findings?
55 Yes ☐ Unclear

Describe mechanism in detail

Pre-existing factors or condition aggravated (not previously supplied)

Page 2 Treatment plan

has been reviewed again on 06/10/2024

☐ No further treatment

Patient C: 1st
 Patient received treatment from 08/09/2024 to 09/10/2024
 Treatment with time away from this environment
 I have received medical medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals to Diagnostic ☐ Allied Health ☐
Specialist ☐ Psychiatrist ☐

☐ This certified unit does not prevent a return to pre-injury duties. ☐ If suitable duties available

☐ Capacity for work reduced (see list of three options) ☐ No functional capacity for any type of work until

Complete the box section if you certified no functional capacity for any type of work
If no functional capacity, state why? If no capacity for more than 7 days, the insurer may contact you to obtain more information)

Estimated time to return to full duties	Estimated time to return to some form of work duties	Estimated time to return to full duties
unknown	unknown	unknown

Part E - Functional ability

(Optional for emergency medical technicians/practitioners. Nurse practitioners not to complete.)

No changes since last session

Not allowed at work or home

Functional class (patient's usual functional ability)	Functional class affected by injury/condition?	What patient can do (if "yes" is selected)
Cerebral function should be based on what CAN be done, NOT available duties. Consider what the patient can do, what the patient can't do, what the patient can't do, what the patient can't do. Functional class (patient's usual functional ability) Functional class affected by injury/condition? What patient can do (if "yes" is selected)	Functional class affected by injury/condition? What patient can do (if "yes" is selected)	What patient can do (if "yes" is selected)

Parameter	Mean	SD	Min	Max
Lower limb	1.0	0.1	0.8	1.2
Upper limb	1.0	0.1	0.8	1.2
Hand function	1.0	0.1	0.8	1.2

	Yes	No
Spinal function	10	10
Cognition/psychosocial functioning	10	10
Division 13, 1988	10	10

Operating as a	No
Operating machine/heavy vehicle	No
Manual tasks	No
Other	No

Part F - Rehabilitation at work - return to work plan

Other considerations or factors that may affect recovery (the nature and range of workplace support)

☐ I require a suitable duties program to be provided to me for approval
I have discussed injury requirements and return to work with the patient and

☐ Insurer ☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (for use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name: Peter Hawes
Practice/Hospital: Our Medical Ashmore
Email: admin@ourmedicalashmore.com.au
Phone: 0756475600 Date: 08/09/2024

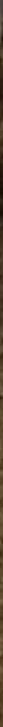
Postal Address
506 Olsen Avenue, Astmore
OLD 4214

Signature 

Further information www.worksafe.qld.gov.au/medicalsupport
All enquiries medical@nsw.practitioner.patient.employer@1300.361.123

Under the Workplace Compensation and Rehabilitation Act 2008 (the Act), the workplace compensation insurer is authorised to collect the information in this form to prevent the insurer's application for compensation. The information collected in this form may be disclosed to the relevant workers' compensation insurer, medical or allied health providers in any other state or territory, compensation authority in any jurisdiction. The consent may be withdrawn by the insured, and the insurer may request the relevant health providers or other medical, allied health or rehabilitation provider, should apply. This form was approved by the Privacy Commissioner (last updated 30 March 2010).

100% of the Act. © State of Queensland (Queensland Treasury) 2008



Photograph of the signed original as held by the Appellant; reproduced at the resolution held.

TAB 3

Email from the practice of Dr Ravikumar Bangalore Krishnaiah to the Appellant, 24 October 2024 at 11:45 am, "Medications"

Where the Respondent holds it

Respondent's amended List of Documents item 9; Appellant's List item 22.

Pages included

Page 1, the email as forwarded from the Appellant's mailbox on 25 November 2024, bearing the original header of 24 October 2024 11:45 am.

Pages omitted, and why

Page 2 (the continuation of the sender's signature block and a list of helpline numbers). Not relied upon.

Schedule row 3 states what this document is, and is not, relied upon for.

Psychiatrist Fw: Medications

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Zach Joubert zjoubert@stonegroup.com.au

Sent: Monday 25 November 2024 at 13:36

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From: Reception

[<Reception@mindandmemoryservice.com.au>](mailto:Reception@mindandmemoryservice.com.au)

Sent: Thursday, October 24, 2024 11:45 am

To: coryshepherd1@hotmail.com

[<coryshepherd1@hotmail.com>](mailto:coryshepherd1@hotmail.com)

Subject: Medications

Hi Cory,

As mentioned, you are suffering from psychological injury of Major Depressive Disorder with anxiety state and obsessive preoccupation about work cover claim and perceived injustice to an extent that your selfcare and daily activities have been neglected.

Please increase Fluoxetine to 2 capsules from today and take Seroquel 25 mg half a tablet for 3 nights and increase to 1 tablet at night

Both these medications helps you to restore basic needs- sleep, eating and routine as well as reduction in obsessive rumination and preoccupation.

I will see you in 3-4 weeks time

Kind regards

Ravi

Dr Ravikumar Bangalore Krishnaiah

TAB 4

Report of Dr Krishnaiah, Mind and Memory Service, 13 February 2025, prepared for QSuper

Where the Respondent holds it

Respondent's amended List of Documents item 10.

Pages included

All four pages. The report is reproduced whole so that nothing relied upon is read out of its context.

Pages omitted, and why

None.

Schedule row 4 states what this document is, and is not, relied upon for.

This information was disclosed for Qsuper and not for medico-legal use. If a patient or any third party requests a copy, please direct them to our office. This message contains confidential information and is intended only for the individual named. If you are not the named addressee, you should not disseminate, distribute or copy this document.

Please notify the sender immediately if you have received this document by mistake and delete that from your system.

Louise Ings
Claims Manager

13-Feb-2025

RE: Report for Mr Cory Shepherd (11-Jan-1991)

104/158 Scarborough Street
Southport QLD 4215 0422 438 627

Many thanks for reaching out for the report. I have undertaken clinical assessment, reviewed materials presented to me regarding his unfair dismissal, workplace conflicts, alleged bullying, harassment and Workcover matter. Further obtained collateral information from his mother independently. Based on above, I am able to provide information on his mental health symptoms, working diagnosis and opinion on impact of his mental health on his daily functioning and occupational impairment. **Mr Cory Shepherd suffers from Major Depressive Disorder with anxious distress (DSM 5- 296.23).**

Mr Cory Shepherd presented with workplace stress stemming from issues with management and rostering at Queensland Health. He reported financial difficulties due to withheld pay, relationship strain, sleep disturbances, and mood changes with difficulty concentrating. These issues began approximately one year ago when a new manager was appointed. After raising concerns about rostering, fatigue management, and workplace policies, he experienced changes in work conditions including shorter breaks, more night shifts, and reduced penalty rates. His pay was withheld or delayed for up to five months at a time, leading to significant financial stress and strain on his relationship.

Cory presented as articulate and engaged during the consultation. Appeared physically drained and showed signs of neglecting personal hygiene, reporting a decrease in his usual self-care routine. He was anxious throughout the interview with intermittent intense sweating, shaking and hesitant speech. Subjectively he described mood as low, rating it 3/10. Affect was observed to be restricted with anxious distress. Expressed feelings of worthlessness and dehumanisation related to his current employment situation and financial stress. Thought Form and Content: Thought form was coherent and goal-directed. Thought content was preoccupied with workplace issues, including perceived unfair treatment, financial stress due to withheld pay, and the ongoing legal proceedings. Demonstrated rumination about these issues, which appeared to dominate his cognitive processes. There was no evidence of perceptual disturbances or hallucinations reported or observed during the consultation. He was oriented to time, place, and person, however was not able to sustain focus on conversation, kept repeating the same issues despite gentle nudging to relevant questions, evident of obsessive nature to his thoughts affecting his cognitive functions. He reported difficulties with concentration and literacy during periods of high stress at work. No formal cognitive assessment was conducted during this consultation. Cory demonstrated partial insight into his current mental state and the impact of workplace stress on his overall functioning. Judgement appeared impaired due to his mental state.

Risk Assessment: No current suicidal ideation or plan reported. Protective factors include a supportive partner and willingness to seek help. However, ongoing financial and occupational stressors may potentially increase risk in the future. There was no current suicidal ideation reported, and no evidence of perceptual disturbances or thought disorder. Insight was assessed as partial.

The severity of his symptoms has significantly impacted his occupational functioning, leading to his current unemployment. Social functioning has been impaired, particularly in his relationship, and he reported reduced self-care with a decreased hygiene routine. Cognitive difficulties, particularly with concentration and working memory.

The treatment plan includes continuing fluoxetine 20mg daily with monitoring for side effects and efficacy. Engagement in self-care activities and stress management techniques has been encouraged, and a referral for psychological therapy (CBT) is being considered. He started treatment with Fluoxetine which was increased to 3 capsules per day and night-time Quetiapine. Consideration was given to adding a low-dose antipsychotic-quetiapine 25mg nocte, for rumination and sleep disturbances.

1. Please detail the impact of the condition on ability to engage in daily activities (such as self-care, domestic activities, driving and community engagement).

Cory's condition has significantly impacted overall daily functioning, with notable effects on occupational performance, financial stability, personal relationships, and self-care routines. The stress and anxiety related to the workplace issues appear to have permeated multiple areas of Cory's life, leading to a marked decrease in his quality of life and ability to engage in normal daily activities.

Cory's workplace stress and related issues have significantly affected various aspects of his daily functioning:

- a. Occupational functioning: Currently not working due to ongoing workplace issues and disputes.
- b. financial management: Experiencing financial difficulties due to withheld or delayed pay, impacting overall quality of life.
- c. Relationships: Strain in current relationship due to financial stress, with partner feeling like a "sugar mommy".
- d. Self-care: Reported decline in general hygiene routines, no longer maintaining daily shower habits as previously done. He has been experiencing sleep disturbances, initial insomnia, obsessive rumination, fear driven thoughts, extensive researching, and planning on fact findings for work-related issues including reading up on policies, procedures, and legislations. His concentration was impaired, reported becoming "illiterate" due to stress, affecting ability to communicate effectively. He has been feeling flat in his mood, varied with unexplained irritability, self-directed anger, and feelings of worthlessness. He stopped spending time with his partner, friends and became socially withdrawn. He was unable to engage in previous interests and activities, suggesting a loss of enjoyment in previously pleasurable pursuits. He stopped exercising and has been neglecting his nutritional needs. His self-confidence and self-esteem had been low to an extent that he started to doubt his own values, beliefs, and ability to make his own decision.

2. Are there any other non-medical factors influencing capacity to work? If yes, please provide brief details.

Yes, there are several non-medical factors influencing capacity to work. Firstly, financial stress due to withheld or delayed pay for up to 5 months at a time has significantly impacted overall wellbeing and ability to meet basic needs. This has led to relationship strain and feelings of worthlessness. Ongoing workplace dispute and legal proceedings, including WorkCover claim and industrial relations review, have consumed his whole life severely affecting his functioning. The prolonged nature of these processes is contributing to rumination and preoccupation with workplace issues. His loss of routine and daily structure had changed since stopping work, disrupted normal coping mechanisms and self-care practices. This has led to a decline in personal hygiene habits and engagement in previously enjoyed activities. His perceived discrimination and unfair treatment in the workplace have eroded self-esteem and confidence in professional abilities. This may impact future job-seeking behaviours and workplace interactions. Cory had limited support from his union representatives has left feeling isolated and unsupported in navigating complex workplace issues, potentially exacerbating stress, and anxiety.

3. What is the current treatment plan and effectiveness of this treatment? Include type of

treatment, frequency, duration, starting/ending dates or ongoing (timeframes).

Current Treatment Plan:

Pharmacological Medications:

1. Fluoxetine 20mg daily, gradually increased to 3 in the morning, response has been gradual in nature.
2. Addition of low-dose antipsychotic -quetiapine 25mg nocte for rumination and sleep, helped him to switch off from the obsessive thoughts

3. Vyvanse 30 mg once a day started recently to manage his treatment resistant depressive state and cognitive impairment

Psychological treatment:

Cory's condition would benefit from regular psychological therapy (CBT) sessions and trauma focused interventions. I will be exploring his willingness to engage in therapy sessions.

Regular follow-up review scheduled to monitor progress, review mental state, and optimise treatment.

4. Please detail how the claimed condition impacts capacity to engage in the duties required of their own occupation:

a. Current capacity for work:

In my opinion Mr Shepherd has complete incapacity to undertake duties required for his own occupation.

b. Current barriers for returning to work within their own occupation:

Ongoing symptoms of severe nature of Major depressive Disorder and significant functional decline

c. Recommendations or support that can be made to assist in returning to work:

- Continue current medications review and optimisation required based on the progress
- Engage in psychological therapy (CBT) to manage symptoms and develop coping strategies
- Implement stress management techniques and self-care activities
- Regular psychiatric follow-up to monitor progress and adjust treatment as needed

d. Long-term barriers:

The duration of these barriers is unclear at this stage. Ongoing treatment and support may help address the current issues, but the outcome of the workplace dispute and legal proceedings could potentially have long-term impacts on the individual's ability to return to their previous occupation. Further, focusing on recovery is key to build psychological capacity to return to work with appropriate focused interventions to determine work-readiness.

5. Please briefly outline current Return to Work goal. If unclear, please provide an indication of a suitable time to review.

Given the complex workplace issues, ongoing WorkCover claim and disputed unfair dismissal proceedings and impaired decision-making abilities it is unclear at this stage the work goal and return to work. I will further update on this after next consultation. Please note that premature exposure to the workplace is more likely result in significant deterioration in his mental health and increased risks of suicide.

6. QSuper would like to partner with you to enable a safe and sustainable return to work, when medically suitable. QSuper can aid via Occupational Rehabilitation to help plan and facilitate suitable return to work goals. Occupational Rehabilitation can also provide strategies to help uplift work-readiness and provide additional support to Cory Shepherd through their return-to-work journey.

a. Is the engagement of an occupation rehabilitation provider likely to assist in facilitating a return to work in the next 1–2 months?

No, the engagement of an occupational rehabilitation provider is not likely to assist in facilitating a return to work in the next 1-2 months due to ongoing symptoms, impaired decision-making ability, and judgement.

b. If no, advise why and when you consider this assistance would be appropriate, or alternatively what strategies or support is being or could be utilised to enable a successful return to work?

The assistance of an occupational rehabilitation provider would not be appropriate at this time due to the following reasons:

1. Current mental health status: Cory is experiencing significant psychological distress due to multiple life stressors, including relationship breakdown, job loss, and bereavement.

2. Ongoing legal issues: Corey is involved in unfair dismissal proceedings with his employer, Queensland Health. This unresolved situation may complicate any return-to-work plans.

3. Need for stabilisation: The current focus is on stabilising Cory's mental health through medication management and psychological interventions

4. Recommendation to defer major decisions: I have advised Cory to defer in making decisions regarding work settlement or legal matters until his mental state improves.

Occupational rehabilitation assistance would be more appropriate once Cory's mental health has stabilised and the legal issues with his former employer have been resolved. This may take several months. Current strategies and support being utilised to enable a successful return to work in the future include:

1. Medication management: Restarting Vyvanse 30 mg daily to assist with focus and cognitive function, alongside continuing Fluoxetine for mood symptoms.

2. Psychological support: Ongoing counselling with Catherine, focusing on coping strategies and emotional regulation and focused CBT/trauma informed therapy

3. Lifestyle interventions: Implementation of a strict daily routine, including self-care activities and sleep hygiene.

4. Regular psychiatric review: A follow-up appointment is scheduled in 4 weeks to assess response to medication changes and routine implementation.

Once Corey's mental health has stabilised and legal issues are resolved, engagement with an occupational rehabilitation provider could be considered to facilitate a gradual return to work.

7. Do you have any additional comments to add not covered in the above?

Cory described himself as detail-oriented, perfectionistic, and having high standards. He tends to hyperfocus on tasks and can become consumed by issues he perceives as unjust. Cory's mother described him as clever, talented, and determined. She reported feeling undervalued and frustrated with perceived inconsistencies in policy implementation and described as having a strong work ethic. However, he struggles with assertiveness and decision-making, particularly in workplace and relationship contexts.

Please feel free to contact me on 0756545580 if you have any questions.

Yours sincerely,



Dr Ravikumar Bangalore Krishnaiah

MBBS, MRCPsych (UK), CCT (General Adult), CCT (Old age/Dementia), CCT (Addiction), FRANZCP, MBA (UK), AFRACMA, MPH (UNSW)

Provider number: 473280RT/473280QF

Consultant General Adult/Older person and Addiction Psychiatrist, Specialist in Adult Neurodevelopmental Disorder and Dementia

Expert in management of ADHD/ASD/Learning Difficulties/Acquired Brain injury and Dual diagnosis and complex case management

TAB 5

Clinical records of Dr Krishnaiah from 24 October 2024

Where the Respondent holds it

Offered by the practice on 5 September 2026 and requested. Not yet received.

Pages included

None at the date of service. The records will be served on receipt, and any direction required for reliance on them will be sought.

Pages omitted, and why

Not applicable.

Schedule row 5 states what this document is, and is not, relied upon for.

TAB 6

Employee Capability Checklist completed by Dr Day Hong Ma, 3 July 2026

Where the Respondent holds it

Not on the Respondent's list. Served with this bundle.

Pages included

All five pages.

Pages omitted, and why

None.

Schedule row 6 states what this document is, and is not, relied upon for.

Employee Capability Checklist – Cory Shepherd

*** Please note that a separate medical certificate is also required to support the completed Capability Checklist.*

Information provided in the Capability Checklist below will assist our Organisation to determine appropriate tasks in the event that the identified worker is cleared for suitable duties. Please ensure that all sections of the Capability Checklist are completed in full.

I have examined Cory Lea Shepherd (DOB 11 January 1991) and consider that they are diagnosed with the following condition:

Major Depressive Disorder with anxious distress (DSM-5 296.23) — work-related psychological injury, onset 18 June 2024.

- ☒ I have viewed the worker's Position Description (attached — Administration Officer, Switchboard Services (AO3), Logan Hospital) and/or
- ☒ I am aware of the requirements of their role

It is my opinion that they are fit to resume:

- ☐ Normal duties
- ☒ Modified duties
- ☒ Reduced hours (hours/days)
- ☐ Workplace Performance Discussions

Modified duties relate to:

- ☒ Tasks performed
- ☒ Duration

Hours to be worked

Week 1: 8 hours/day 3 days/week	Week 5: 8 hours/day 3 days/week
Week 2: 8 hours/day 3 days/week	Week 6: 8 hours/day 3 days/week
Week 3: 8 hours/day 3 days/week	Week 7: 8 hours/day 3 days/week
Week 4: 8 hours/day 3 days/week	Week 8: 8 hours/day 3 days/week
(6 shifts per 10-shift fortnight: max 2 weekday day/afternoon + 2 night + 2 weekend shifts — continuation of the reduced pattern Mr Shepherd has sustained since 2025)	

Fit to commence the above from: 3 July 2026 (continuation of existing arrangement)

Date of next review: 28 August 2026 (approximately 8 weeks)

Activity	No capacity	Restricted / As Tolerated	Normal	Details of any restrictions and reasons
COGNITIVE				
Learning new tasks	☐	☐	☒	
Ability to initiate tasks independently	☐	☐	☒	
Understanding instructions	☐	☐	☒	
Carrying out instructions accurately	☐	☐	☒	
Maintain focus/concentration	☐	☒	☐	Concentration reduced during sustained high-stress periods; managed by rostering to lower call-volume shifts.
Executing tasks within a reasonable timeframe	☐	☐	☒	
Working autonomously without supervision	☐	☐	☒	
Prioritising tasks	☐	☐	☒	
Making decisions	☐	☐	☒	
Multi-tasking	☐	☒	☐	Concurrent high-volume demand exacerbates symptoms; suitable on lower-volume shifts.
Memorising information	☐	☒	☐	Working memory affected under stress (treating psychiatrist report 13-Feb-2025).
Working in a high demand environment	☐	☒	☐	Limit weekday day/afternoon shifts — which carry the peak call volume and the majority of complaints — to no more than 2 per fortnight; remaining shifts rostered to the lower-volume weekend (2) and night (2) shifts.
RELATIONAL				
Communicating with internal/external customers	☐	☐	☒	
Working in a team environment	☐	☐	☒	
Handling conflict	☐	☒	☐	No complaint-handling duties: complaints received at the Switchboard are to be logged and redirected to management/the dedicated complaints area, not actioned, resolved or absorbed by Mr Shepherd.
Supervising others	☐	☐	☒	Not a requirement of the AO3 role.
Interacting appropriately with others	☐	☐	☒	
Do you foresee any existing stressors that could impact on a return to substantive role? If so, please provide additional comments:				Yes. (1) Complaint-handling exposure at the Switchboard; (2) being held accountable, and blamed, for failures that are the responsibility of others across the hospital, regardless of where the fault lies; (3) rostering and fatigue-management practices; (4) unresolved workplace matters involving line management. Recommend return-to-work communications and this checklist be managed through the Injury Management Team Human Resources

Medications

Is the worker currently taking any medication?

☒ Yes

☐ No

Please outline any side effects from medication currently being taken that may impact on a person's functional capacity or task performance (for example drowsiness, dizziness, confusion, impaired concentration, fatigue, slurred speech):

Current medications:

1. Fluoxetine 60 mg mane (3 × 20 mg capsules).
2. Lisdexamfetamine (Vyvanse) 30 mg mane — for cognitive symptoms and focus.

Quetiapine (Seroquel) 25 mg nocte, previously prescribed for rumination and sleep, has been ceased.

Relevant effects on task performance: maintaining a regular sleep routine remains central to symptom management following cessation of the night-time medication. This supports (a) limiting night shifts to no more than 2 per fortnight, (b) predictable rostering published in advance, and (c) a minimum 10-hour break between rostered shifts consistent with the applicable Award and fatigue risk management standards. Residual symptoms include reduced concentration and working memory under sustained high-demand load and fatigue sensitivity.

Additional Comments:

1. Please elaborate on the current medical condition Mr Shepherd is suffering from?

Major Depressive Disorder with anxious distress (DSM-5 296.23), a work-related psychological injury with onset 18 June 2024. Treatment is ongoing (fluoxetine, lisdexamfetamine) under psychiatric care. Residual symptoms most relevant to work capacity are reduced concentration and working memory under sustained high-demand load, fatigue sensitivity, and symptom exacerbation on exposure to the identified workplace stressors (complaint handling; being held accountable and blamed for the failures of others; unpredictable rostering).

2. In your opinion do you feel that Mr Shepherd's current condition is temporary, episodic or permanent in nature? What is the expected duration of the current restrictions?

Episodic, with symptom exacerbation linked to stressor exposure. The restrictions below are expected to remain necessary for at least the next 3–6 months, subject to treatment response and 8-weekly review.

3. Based on your current assessment, what work capacity (including number of shifts, hours, and type of duties) would best support Mr Shepherd's recovery while minimising risk of exacerbation?

Six 8-hour shifts per fortnight (of the ten rostered in a full-time fortnight), comprising:

(a) No more than two weekday day/afternoon shifts. These shifts carry the peak call volume and the majority of complaints — the identified stressors. Retaining two per fortnight maintains graded exposure to the full role and preserves work capacity; the cap prevents multiple high-load shifts accumulating within a fortnight, which is the pattern associated with symptom exacerbation.

(b) Two weekend day shifts. Weekend call volume and complaint exposure are materially lower while the duties are identical, allowing the same work to be performed at an intensity consistent with Mr Shepherd's current concentration and working-memory capacity, and preserving a normal sleep-wake cycle.

(c) No more than two night shifts at this stage. Night shifts are low in volume and complaints, but a stable sleep routine is central to symptom management following cessation of the night-time medication; capping nights protects that routine and limits cumulative fatigue. This cap is suitable for reassessment at review.

This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation. Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below.

4. Are there any specific workplace modifications or supports you would recommend (e.g. reduced workload, predictable rostering, supervision level, reduced exposure to stressors) to assist Mr Shepherd during this period?

(a) No complaint-handling duties — complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd; (b) predictable rostering published in advance, with a minimum 10-hour break between shifts consistent with the Award and fatigue risk management standards; (c) return-to-work coordination and correspondence through the Injury Management team, Human Resources; (d) access to EAP (TELUS Health) as an available support; (e) review in approximately 8 weeks.

Please complete this form and return to the Injury Management team, Logan & Beaudesert Health Service, Human Resources by email lbh_injurymanagement@health.qld.gov.au

Please include your practice details/stamp here

Signature

Date



3-7-26.

**MY DOCTORS CLINIC
OPEN 24 HOURS**

Dr Day Hong Ma

MBBS,FRACGP,FACRRM Provider No: 4696842K

16/3221 Surfers Paradise Blvd

Surfers Paradise, QLD 4217

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TAB 7

Review Decision 69983, reasons for decision dated 24 October 2024, pages 17, 26 and 27 of 28

Where the Respondent holds it

Respondent's amended List of Documents item 4. Contents admitted by the Respondent on 18 February 2026.

Pages included

Page 17 (the medical evidence and the finding that employment was a significant contributing factor), and pages 26 and 27 (the findings on management action and the conclusion).

Pages omitted, and why

The remaining 25 pages, which the Respondent holds and which are not relied upon for the purposes stated in the schedule.

Schedule row 7 states what this document is, and is not, relied upon for.

Reasons for decision

The work capacity certificate of Dr Hawes dated 1 July 2024 indicated your mechanism of injury was consistent with his clinical findings and there was no pre-existing factor or condition. This was maintained in all later work capacity certificates.

According to the communications report, in a conversation with WorkCover on 2 September 2024, Dr Hawes stated that work events were the sole cause of your psychological injury.

Having regard to the medical evidence, I am satisfied your employment was a significant contributing factor to the psychological injury.

It is now necessary for me to determine whether your injury has arisen out of or in the course of management action, and if so, whether management action is found to be reasonable and taken in a reasonable way.

Did the injury arise out of or in the course of management action?

The term 'management action' is not defined in the Act. It has been interpreted by the Courts broadly and will vary depending on the circumstances of the case. The action must relate to action taken by or on behalf of the employer in connection with the worker's employment.

In *Avis v WorkCover Queensland*², the Industrial Court held that the phrase 'arising out of' involves a causal or consequential relationship between the management action and the injury, but does not require a direct or proximate relationship.

Factor 2: Management action taken in response to your complaint

Submitting complaints regarding aspects of your employment is connected to your employment. The complaint was reviewed by the Ethical Standards Unit, and then by Human Resources. These departments have the power to take action in response to your complaint on behalf of the employer. I find the steps taken by them were taken on behalf of the employer in connection with your employment and therefore amounted to management action.

Factor 3: Withholding of pay

The pay issues arose as a result of Ms Taylor refusing your claims for payment in specific situations. Ms Taylor was your direct manager and thus in a superior position, which provided her with the power to manage your employment. Receiving payment for your work is directly

² [2000] QIC 67

Reasons for decision

In your response dated 9 August 2024, you stated you were expected to finish on 17 March 2024 at 11:00 pm and start again on 18 March 2024 at 6:00 am. This meant you received only a 7-hour break between the shifts, which did not include travel time. If you factored in travel time, your break between shifts would have been less than 5 hours. You reiterated it was unreasonable for Ms Taylor to apply the 8-hour agreement you signed earlier in this situation.

The response of the employer dated 6 September 2024 submitted that during pay period 20, commencing on 4 March 2024, you were scheduled for 3 night shifts with appropriate rest days allocated. You arrived 15 minutes late on 17 March 2024. The employer wrote that records indicate Ms Taylor took appropriate action by shortening your shift, but this resulted in you being remunerated for the 30 minutes you did not work. The employer did not mention your shift on 18 March 2024. They also confirmed you had not formally rescinded your agreement for only an 8-hour break, which you signed on 17 June 2020, and the change of your contracts and adjustments in working hours did not invalidate this agreement. This meant the agreement remained current. In their response, the employer included an extract of the Hospital and Health Services General Employees (Queensland Health) Award, which stated that employees must be provided with a break of not less than 10 hours between the termination of one shift and the commencement of another shift, and 8 hours applied instead of 10 only in specific circumstances.

In considering the evidence, I find there was uncertainty between you and the employer regarding whether the 8-hour agreement continued to apply. However, it is not necessary for me to consider whether the employer's stance on its application was reasonable. The break between the shift on 17 March 2024 and 18 March 2024 equated to 7 hours. Even if you were allowed to leave early on 17 March 2024 as suggested by the employer, you left a maximum of 30 minutes early, which meant you still did not receive a minimum 8-hour break. Based on this, I find the rostering of these two shifts amounted to unreasonable management action given that it was in direct contradiction to the award and the 8-hour agreement.

Conclusion

After considering the evidence available at review, in relation to the factors for determination, I have determined:

- you sustained a personal injury of a psychological nature
- your injury arose out of employment, to the extent that it arose out of factors 2, 3 and 4, where employment was a significant contributing factor
- factors 2 and 3 amounted to reasonable management action taken in a reasonable way

Reasons for decision

- factor 4 amounted to unreasonable management action.

In the decision of *Delaney v Q-COMP*⁵ the Industrial Court held that in determining the issue of whether management action was reasonable and taken in a reasonable way, a global evaluation of management actions must be made. In *Delaney*, it was held that when management actions are joined by subject matter, time and personality, the actions are entitled to be evaluated on a global basis.

I note that since two out of the three causative factors amounted to reasonable management action taken in a reasonable way, this means your psychological injury mainly arose out of such management action. Further, factor 4 was initially only substantiated to the extent of one incident and it was only the one incident which amounted to unreasonable management action. Therefore, I find overall your psychological injury arose out of reasonable management action taken in a reasonable way and section 32(5) is consequently enlivened.

Summary

I therefore conclude that the provisions of section 32(5) of the Act exclude your psychological condition from the definition of 'injury' within section 32(1) of the Act.

Therefore, I have determined that you did not sustain an 'injury' in accordance with section 32 of the Act and you do not have an entitlement to compensation.

Appeal rights

If either party disagrees with this decision then either party may appeal to the Queensland Industrial Relations Commission in Brisbane. Either party has 20 business days from the date of receipt of this decision in which to lodge an appeal. A copy of the notice lodged with the Commission must also be served on the Workers' Compensation Regulator within 10 business days.

For information about the appeal process please contact me or refer to:

<https://www.worksafe.qld.gov.au/claims-and-insurance/compensation-claims/appealing-a-review-decision/guide-to-appealing-a-review-decision>.

⁵ [2005] QIC 11; 178 QGIG 197